SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Temozolomide with short-course radiotherapy, then adjuvant treatment

Adults older than 65 with newly diagnosed glioblastoma who are not suitable for the standard radiotherapy regimen with temozolomide, under NCCP 00461. · NCCP 00461 · version 3 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This plan combines temozolomide with a shorter course of radiotherapy, followed by a break and adjuvant temozolomide, to help control newly diagnosed glioblastoma. NCCP identifies this particular indication as unlicensed; your team explains the evidence, alternatives and local consent process.

Schedule
Take temozolomide daily for 21 days during short-course radiotherapy, including weekends as prescribed. After a four-week break, take temozolomide on Days 1–5 of each 28-day cycle for up to six adjuvant cycles.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Temozolomide
ORAL CHEMOTHERAPY CAPSULE

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea / vomiting and appetite loss
- ☐ Fatigue, headache and constipation
- ☐ Liver injury, including delayed injury
- ☐ PJP and other opportunistic infections
- ☐ Rash and rare severe skin reactions
- ☐ Fertility effects and harm to a pregnancy
- ☐ Rare serious bone-marrow disorders

Radiotherapy
TREATMENT FOCUSED ON THE BRAIN TUMOUR AREA

- ☐ Fatigue and scalp reaction
- ☐ Hair loss in the treated area
- ☐ Swelling and neurological symptoms
- ☐ Possible delayed effects on thinking and function

Discuss treatment intent, evidence and alternatives; phase-specific schedules; blood and organ monitoring; infection, nausea, fertility and pregnancy precautions; practical support and any unlicensed use. Keep the separate radiotherapy consent and medication plans where applicable.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00461 v3.

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